

Understanding Antidepressants (SSRIs & SNRIs)

General guidance on how SSRI and SNRI antidepressants work, common side effects, and what to expect during treatment.

What are SSRIs and SNRIs?

Selective Serotonin Reuptake Inhibitors (SSRIs) and Serotonin-Norepinephrine Reuptake Inhibitors (SNRIs) are the most commonly prescribed antidepressant medications, used for depression, anxiety disorders, OCD, and panic disorder. They work by increasing the availability of serotonin (and, for SNRIs, norepinephrine) in the brain, which helps regulate mood over time.

What to expect

- Full effect usually takes 4-6 weeks, though some improvement in sleep or anxiety may be noticed earlier
- An initial period of mild side effects (nausea, jitteriness, headache) often settles within 1-2 weeks
- Your psychiatrist may adjust the dose based on your response and side effects
- Treatment is usually continued for 6-12 months after symptoms improve, to reduce relapse risk

Common side effects

- Nausea or stomach upset, usually early and temporary
- Headache or mild dizziness
- Sleep changes — some antidepressants are more activating, others more sedating
- Reduced sexual interest or response, which can persist and should be discussed with your psychiatrist

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Important safety notes

- Do not stop abruptly — stopping suddenly can cause discontinuation symptoms (dizziness, flu-like feelings, irritability); doses should be tapered under medical guidance
- Tell your psychiatrist about all other medications and supplements, since some combinations increase serotonin syndrome risk
- Report any new or worsening suicidal thoughts immediately, especially in the first few weeks or after a dose change
- Avoid combining with alcohol or recreational drugs without discussing this with your psychiatrist

When to seek urgent help

Seek urgent medical attention for agitation, high fever, muscle rigidity, rapid heart rate or confusion (possible serotonin syndrome), or for any new or worsening thoughts of self-harm.

This material is for patient and family education only and does not replace individualized medical advice. For a personal diagnosis or treatment plan, consult Dr. Kushal Kharel or a qualified mental health professional.

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